

Hospital Fort Bend Fourth Floor: Options for the FY2028 Capital Plan

Tallowbrook Health | Office of the President, Hospital Fort Bend | 2026-09-12 | v0.9 (draft for executive review)

Decision memo, draft for executive review, September 12, 2026. From Janelle Brooks, President, Tallowbrook Hospital Fort Bend, with Strategy & Communications and Facilities & Plant Operations. To the executive team.

Request

Approve \$350,000 for schematic design covering both build-out scopes described below, so that cost estimates can be refined before the board decides on the fourth floor in February 2027 as part of the FY2028 capital plan.

This memo does not recommend an option. It sets out the evidence and the choices so the executive team can decide what to take to the board.

The hospital today

Hospital Fort Bend has 112 licensed beds, of which 96 are staffed, a 24-hour emergency department with 24 treatment spaces and six operating rooms. The fourth floor was built as shell space sized for 36 beds.

Obstetric patients are referred to Medical Center West, where Women's & Infant Services is offered. None of the options below adds obstetrics.

- ICU: 12 beds
- 2 North Medical-Surgical: 32 beds
- 3 North Medical-Surgical/Telemetry: 30 beds
- 3 South Surgical: 22 beds

Demand

FY2026 admissions were 5,893, up 7.2% on FY2025, with 24,616 patient days and an average length of stay of 4.18 days. Average occupancy of staffed beds rose from 66.0% to 70.3%.

ED visits rose 5.6% to 43,112, and the hospital performed 4,589 surgical cases.

Growth has continued. In July and August 2026 admissions were 1,006 against 948 a year earlier, and ED visits 7,677 against 7,254.

Where the pressure is

Average occupancy understates pressure on the medical units. From February to August 2026, midnight occupancy including observation patients was 76.4% for the hospital, above Medical Center West at 75.6%.

- 2 North: 81.8%
- 3 North: 80.5%
- ICU: 73.2%
- 3 South: 64.6%

Market outlook

Planning assumes continued population and household growth in the Fort Bend service area.

The strategy team refreshes demand projections each fall before the capital plan; this year's refresh was completed in August and will be updated again before the board meeting.

Options

Preliminary estimates are order of magnitude and will change with design.

- A. Defer. No capital in FY2028; manage growth through length of stay, flow work and use of 3 South capacity.
- B. Build 18 medical-surgical beds as phase 1 in FY2028. Preliminary estimate \$14.5 million (about \$0.81 million per bed); the remaining shell stays available.
- C. Build all 36 beds in FY2028. Preliminary estimate \$26.0 million (about \$0.72 million per bed).

Considerations

- Option A carries the risk that the medical units run above 80% more often as volumes grow, increasing ED boarding at Fort Bend.
- Option B costs more per bed and needs a second construction period on an occupied floor later.
- Option C gives the lowest cost per bed but adds staffing and operating costs before demand may need all 36 beds.
- 3 South ran at 64.6%; any option should be tested against using some surgical beds for medical patients.
- New beds must be staffed. Central Staffing float pool growth at Fort Bend should be reflected in the staffing plan for any build-out.

Next steps

If design funding is approved, schematic design would run October to December 2026, with refined estimates and updated demand projections in January 2027 ahead of the February board meeting.